OLLIE HINKLE
HEART FOUNDATION

OLLIE'S
BRANCH
Extending Peace of Mind

Caring Beyond the Diagnosis:
A Trauma-Informed Therapist's Guide to Pediatric Heart Disease

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Purpose of This Guide

This guide is designed to help mental health providers understand the lived experience of individuals and families affected by congenital and acquired heart disease.
It is not intended to make you a medical expert. Instead, it provides foundational knowledge so you can better understand:

- The emotional and psychological impact of heart disease
- Common medical experiences your clients may reference
- How trauma, uncertainty, and chronic illness shape development and family systems

Every heart journey is different. This guide offers context—not assumptions.

About Ollie Hinkle Heart Foundation

Ollie Hinkle Heart Foundation supports children and families impacted by childhood-onset heart disease through care coordination, mental health support, and community-driven programs.

This guide is provided to the Ollie's Branch Therapist Community to ensure families receive informed, compassionate mental health care.

What Mental Health Providers Should Know

- Congenital heart disease (CHD) is a **lifelong condition**, not something that is "fixed" with surgery
- Many children and caregivers experience **medical trauma**, even if they don't use that language
- Families often live with **chronic uncertainty** as conditions can change quickly
- Caregivers may experience:
    - PTSD symptoms
    - chronic stress and hypervigilance
    - guilt, grief, and identity loss
- Children may:
    - appear resilient but struggle internally
    - experience developmental or learning differences
- Medical environments can create **lasting triggers** (alarms, procedures, separation, authority figures)

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Peer Support & Community

Clinical Lens:
Many heart families seek connection with others who understand their experience, but may limit themselves to those with the exact same diagnosis—leading to isolation.

Shared lived experience often matters more than matching diagnoses.

Key Concepts

Concept	What It Means	What Therapists Should Know
Peer-to-Peer Support	Connection with other patients or caregivers in the heart community	Reduces isolation, normalizes experiences, and strengthens coping and resilience
Diagnosis Matching	Seeking support only from families with the same specific heart defect	Can unintentionally limit access to support and reinforce isolation
Broader Heart Community	Families across different CHD diagnoses and experiences	Shared themes (uncertainty, trauma, caregiving stress) create meaningful connection regardless of diagnosis
Barriers to Engagement	Fear, comparison, medical complexity, or past experiences	Therapists can gently expand perspective and support safe, gradual connection

In Practice

- Normalize the desire for "someone who gets it"
- Gently reframe connection around **shared experiences**, not exact diagnoses
- Explore safe entry points:
    - small groups
    - moderated communities
    - one-to-one connections
- Be mindful that comparison or overwhelm may initially increase anxiety

Health Disparities & Access to Care

Not all children and families experience heart disease in the same way. Access to diagnosis, treatment, and ongoing care is shaped by systemic and environmental factors.

What This Can Look Like

- Delayed diagnosis or intervention due to limited access to specialty care
- Long travel distances for appointments, surgeries, or follow-up care
- Financial strain related to medical bills, time off work, lodging, and transportation
- Inconsistent access to medications or therapies
- Communication barriers (language differences, health literacy, or unclear medical information)
- Experiences of bias, dismissal, or lack of trust within healthcare settings

Who May Be Most Impacted

- Families from lower-income backgrounds
- Families living in rural or medically underserved areas
- Black, Indigenous, and other families of color
- Families navigating language barriers or immigration-related stressors

Why This Matters for Therapy

These factors shape not only medical outcomes, but also emotional experience.

Clients and caregivers may:

- Carry mistrust of medical systems based on past experiences
- Feel guilt or shame related to barriers outside of their control
- Experience added stress from financial or logistical strain
- Have difficulty following care plans due to access—not motivation

For many families, the burden of managing heart disease is layered on top of existing inequities.

Clinical Considerations

- Avoid assumptions about "noncompliance" or lack of follow-through
- Explore barriers with curiosity and without judgment
- Validate the impact of systemic stressors on the family system

- Be mindful of power dynamics when discussing medical care

Understanding Pediatric Heart Disease

Congenital Heart Disease (CHD)

Congenital heart disease refers to structural abnormalities of the heart present at birth.
These may affect:

- How blood flows through the heart
- How oxygen is delivered to the body
- How hard the heart has to work

CHD is the **most common birth defect**, affecting about 1 in 100 babies.

Why this matters for therapy:

- Often involves surgeries, hospitalizations, and ongoing care
- Families may experience long periods of uncertainty
- Even "repaired" conditions often require lifelong monitoring

Acquired Heart Disease

Acquired heart conditions develop after birth due to infection, inflammation, or other medical factors.
Examples include:

- arrhythmias (irregular heart rhythms)
- cardiomyopathy (heart muscle disease)

Why this matters for therapy:

- May involve sudden diagnosis or medical events
- Can be associated with traumatic experiences (e.g., cardiac arrest)

Medical Experiences That Shape Mental Health

Surgeries & Procedures

Children with heart disease may undergo:

- Open-heart surgery
- Cardiac catheterizations
- Repeated diagnostic testing

Why this matters:

- Even medically "routine" procedures can be traumatic

- Repeated interventions can create fear, anxiety, and loss of control

Intensive Care (NICU, PICU, CICU)
Many children spend time in intensive care units, especially early in life.
Clinical Insight: ICU Experience
Families often describe ICU environments as overwhelming due to:

- constant alarms and noise
- bright lights and limited privacy
- uncertainty about outcomes

Caregivers may delay eating, sleeping, or leaving the room due to fear of missing updates.

Transitions Between Hospital and Home
Children may cycle between stability and crisis.

Clinical Insight: Uncertainty
Families often describe feeling like they are "waiting for the next thing to happen," even during stable periods.

Life at Home
Daily Medical Management
Families may manage:

- multiple medications
- feeding tubes or nutritional challenges
- frequent medical appointments

Therapeutic impact:

- High caregiver burden
- Rigid routines can increase stress

Social & Developmental Impact
Children may:

- miss school frequently
- have limited participation in activities
- feel different from peers despite "looking normal" from the outside

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Social Determinants of Health

Clinical Lens: Social and structural factors significantly shape a family's ability to access, understand, and engage in care.

Barriers to care are often misinterpreted as lack of engagement.

Factor	What It Means	What Therapists Should Know
Access to Care	Ability to access specialized cardiac care, follow-ups, and interventions	Families may travel long distances, experience delays in diagnosis, or miss care due to access barriers
Insurance & Financial Strain	Coverage for medical care, therapies, and medications	Financial stress can be significant; may impact treatment decisions and mental health
Transportation	Ability to get to frequent appointments and hospital visits	Missed appointments or added stress due to unreliable transportation
Employment Impact	Caregivers needing time off work or leaving jobs to care for child	Can lead to financial strain, identity loss, and caregiver burnout
Housing Stability	Stable, safe environment to support medical needs	Instability can complicate care routines, equipment use, and recovery
Health Literacy	Understanding of complex medical information	Families may feel overwhelmed or intimidated; important to use clear, supportive language
Social Support	Access to family, community, or emotional support systems	Isolation is common; lack of support increases caregiver stress and mental health risk

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Neurodevelopment & Neurodiversity in CHD

Children with congenital heart disease are at increased risk of neurodevelopmental differences.

Why This Happens

- Altered blood flow during early development can impact the brain
- Early and/or frequent surgeries and medical interventions can affect neurological outcomes
- Extended hospitalizations can impact early attachment and sensory development

Common Differences

Children with CHD may be more likely to experience:

- ADHD / ADD
- Executive functioning challenges
- Developmental delays (speech, motor, cognitive)
- Learning differences
- Sensory sensitivities
- Social communication challenges
- Anxiety and emotional regulation difficulties

How This Shows Up in Therapy

- Difficulty focusing or sitting still
- Emotional dysregulation
- Frustration tolerance challenges
- Social difficulties
- School-related stress

Important Considerations

- Behaviors are often **neurological, not behavioral choices**
- Medical complexity and neurodiversity often coexist
- Symptoms may be overlooked due to focus on cardiac or other physical care

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Trauma + Neurodiversity Overlap

Children with CHD often experience repeated medical stressors that may mimic or amplify neurodivergent traits.

Therapists should assess for both:

- neurodevelopmental differences
- trauma responses

Support Strategies that May Help

- Use predictable, structured sessions
- Incorporate visual supports
- Adjust pacing and expectations
- Collaborate with schools (IEPs, 504 plans)
- Consider neuropsychological referrals when appropriate

Common Themes in Therapy

- Medical trauma / PTSD
- Anxiety (especially anticipatory anxiety)
- Loss of control
- Identity challenges ("I look healthy, but I'm not")
- Caregiver burnout
- Survivor's guilt (especially in transplant cases)
- Transition to adulthood (increasing independence vs. ongoing medical reliance)

Grief & Chronic Illness Context

- Ongoing, cyclical grief related to living with a lifelong condition (often referred to as **chronic or ambiguous grief**)
- Anticipatory grief before procedures, test results, or potential changes in health status
- Grief tied to missed milestones, physical limitations, or a different life path than expected
- Caregiver grief that coexists with hope, resilience, and daily functioning
- Sibling and family system grief (shifts in attention, roles, and emotional burden)
- Loss and bereavement following the death of a child or loved one within the heart community

Clinical Lens:
Grief in the heart community is not linear or time-bound. It often resurfaces at each medical event, developmental milestone, or transition point.

Therapy Considerations by Setting

Inpatient / Hospital

- High stress and sensory overload
- Limited autonomy
- Acute trauma exposure

At Home

- Caregiver exhaustion
- Sibling dynamics
- Ongoing stress cycles

School

- Missed instruction
- Social challenges
- Invisible illness misunderstandings

Working with Caregivers

Caregivers are often managing:

- chronic fear and uncertainty
- medical decision-making pressure
- grief (expected and ambiguous)

Clinical Insight: Caregiver Experience

Many caregivers report feeling helpless, especially during hospitalizations when they cannot care for their child in typical ways.

Language Matters

Avoid:

- "fixed"
- "normal life now"

Use:

- "ongoing journey"
- "medically complex"
- "unique developmental path"

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Caring Beyond the Diagnosis

Children and families affected by heart disease often demonstrate incredible resilience—but resilience does not mean absence of struggle.

As a mental health provider, your role is not to understand every medical detail, but to:

- validate lived experience
- recognize trauma and uncertainty
- support both emotional and developmental needs

Your understanding can significantly impact how supported these families feel.

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Appendix A: Pediatric Heart Disease Glossary

This glossary is designed as a quick-reference tool for therapists. It pairs common medical terms with what is most relevant clinically in therapy.

How to Use This Glossary

- You do not need to memorize these terms
- Use this as a reference when unfamiliar language comes up in session
- Focus on the **emotional and psychological impact**, not the medical detail

This glossary is intended to support trauma-informed, developmentally aware care for children and families impacted by heart disease.

HEART CONDITIONS

Term	What It Means	What Therapists Should Know
Congenital Heart Disease (CHD)	Heart condition present at birth affecting structure or function	Lifelong condition; often involves repeated medical experiences and chronic uncertainty
Acquired Heart Disease	Heart condition that develops after birth (infection, inflammation, etc.)	May involve sudden onset or traumatic medical events
Ventricular Septal Defect (VSD)	Hole between lower heart chambers	May involve early monitoring or surgery; family may have experienced prolonged stress
Tetralogy of Fallot (TOF)	Complex condition involving four heart defects affecting oxygen levels	Often involves early surgery and visible symptoms (e.g., blue skin), which can impact identity
Hypoplastic Left Heart Syndrome (HLHS)	Underdevelopment of the left side of the heart	One of the most severe forms; requires multiple surgeries and long-term care

PROCEDURES & TESTS

Term	What It Means	What Therapists Should Know
Open-Heart Surgery	Surgery requiring the chest to be opened and use of a heart-lung machine	Highly traumatic experience for both child and caregivers; often repeated
Cardiac Catheterization (Cath)	Minimally invasive procedure using tubes to diagnose or repair heart defects	Still stressful and can involve uncertainty about outcomes
Echocardiogram (Echo)	Ultrasound used to view heart structure and blood flow	Requires stillness; can be distressing for young children
Electrocardiogram (EKG/ECG)	Test measuring heart's electrical activity	Often repeated; can create anxiety due to medical setting

MEDICAL DEVICES & SUPPORTS

Term	What It Means	What Therapists Should Know
Pacemaker	Device that regulates heart rhythm	Child may be aware of device; can impact body awareness and identity
Implantable Cardioverter Defibrillator (ICD)	Device that detects and corrects dangerous heart rhythms	Shocks can be physically and emotionally distressing
Ventricular Assist Device (VAD)	Mechanical pump that supports heart function	Often used in severe cases; may indicate high medical complexity
G-Tube	Feeding tube placed directly into the stomach	Can impact feeding, development, and caregiver stress
NG Tube	Tube inserted through the nose for feeding	Can be uncomfortable and distressing; may create sensory aversions

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CARE ENVIRONMENTS

Term	What It Means	What Therapists Should Know
NICU	Intensive care unit for newborns	Early separation and medical trauma can impact attachment
PICU	Intensive care unit for children	High-stress environment with frequent alarms and interventions
CICU	Cardiac-specific intensive care unit	Repeated admissions common; families often experience chronic stress

CRITICAL CARE SUPPORT

Term	What It Means	What Therapists Should Know
ECMO	Life support machine that takes over heart and lung function	Highly traumatic visual and emotional experience for families
Heart Transplant	Replacement of a failing heart with a donor heart	Can involve complex emotions including fear, guilt, and identity shifts
Dialysis	Medical treatment that removes waste and extra fluid when kidneys are not functioning properly	Often used in critical illness; can signal high medical complexity and prolonged hospitalization, contributing to caregiver stress and trauma

HOME MONITORING

Term	What It Means	What Therapists Should Know
Pulse Oximetry	A device that measures oxygen levels in the blood using a sensor (usually on finger or toe)	Families may monitor frequently; can increase anxiety and hypervigilance around numbers

Daily Weights	Routine tracking of a child's weight to monitor growth and fluid status	Small changes can feel high-stakes; can create stress and fear around feeding and health stability

Appendix B: Adult Congenital Heart Disease (ACHD)

What Therapists Should Know

Adult Congenital Heart Disease (ACHD) refers to individuals born with heart defects who are now living into adolescence and adulthood due to advances in medical care.

This population is rapidly growing, and many will require lifelong care.

Why This Matters Clinically

- CHD is **not cured** in childhood—most individuals require ongoing monitoring and care into adulthood
- Transitions from pediatric to adult care are often **high-risk periods** for both medical and mental health outcomes
- Many adults with CHD carry **longstanding medical trauma** from childhood experiences
- There is often a mismatch between **external appearance and internal health realities**

Common Psychological Themes in ACHD

- Anxiety related to health, procedures, and future uncertainty
- Medical trauma and PTSD symptoms
- Identity challenges ("Am I sick? Am I healthy?")
- Fear of decline or sudden medical events
- Difficulty with independence vs. reliance on caregivers or medical systems
- Survivor's guilt, especially among those with complex conditions
- Depression related to limitations, fatigue, or chronic illness burden

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Developmental & Life Stage Considerations

Adolescence & Young Adulthood

- Transitioning to self-management of care
- Navigating disclosure to peers, partners, and employers
- Risk-taking behaviors vs. medical limitations

Adulthood

- Career planning and employment limitations
- Insurance and financial stress
- Family planning and pregnancy considerations
- Long-term uncertainty about health trajectory

Barriers & Risks

- Loss to follow-up care during transition from pediatric to adult systems
- Limited access to ACHD-specialized providers
- Difficulty finding providers with ACHD expertise, leading to delays or gaps in care
- Receiving incorrect, outdated, or incomplete medical guidance from non-specialized providers
- Underestimation of condition severity due to "looking healthy"
- Social isolation or difficulty relating to peers without chronic illness

Clinical Considerations for Therapists

- Do not assume medical stability = emotional stability
- Assess for both **trauma history and current anxiety**
- Support identity development within the context of chronic illness
- Explore independence while acknowledging real medical constraints
- Encourage connection with others living with CHD across age groups

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Clinical Lens

ACHD clients often live in a space between worlds—not fully identifying as "sick," yet never fully free from their condition. This ambiguity can shape identity, relationships, and long-term mental health.

Therapists play a key role in helping individuals integrate their medical history into a cohesive and supported sense of self.

What Clients Might Say

- "I don't know if I'm sick enough to need help, but I don't feel normal either."

- "People think I'm fine because I look healthy."
- "I've been dealing with this my whole life—I don't even know what's 'normal.'"
- "I'm scared something could happen, even when doctors say I'm stable."
- "I don't want my heart condition to define me, but it impacts everything."
- "I feel behind compared to other people my age."
- "I don't know how to explain this to friends, partners, or employers."
- "I've been through so much medically, but I've never really talked about it."